

Accidental Bowel Leakage (Fecal Incontinence)

WARWIKI

Patient Information · Losing control of gas or stool is common and treatable

Accidental bowel leakage means losing control of gas or stool. You may leak before you reach the toilet, or without noticing. It is much more common than people think, especially after childbirth or with age. It is treatable. There is nothing to be embarrassed about. Telling your team is the first step to getting better.

About This Condition

Holding in stool depends on the **anal sphincter muscles**, the pelvic floor, how firm the stool is, and the nerves that control them. A problem with any of these can cause leakage. It can range from staining or gas to loss of solid stool.

What Causes It

- **Sphincter injury**, often from a severe tear during childbirth or from past anal surgery
- **Loose stool or diarrhea**, which is harder to hold than firm stool
- **Severe constipation**, with watery stool leaking around hard stool
- **Nerve problems**, rectal prolapse (the rectum slipping out), or irritable bowel

Often more than one cause is at work, and several can be improved.

Learn the Terms

Fecal incontinence

Accidental loss of stool. The broader term includes gas.

Anal sphincter

The ring of muscle that holds stool in until you choose to go.

OASIS

A severe (third- or fourth-degree) childbirth tear that involves the sphincter.

Fiber

A supplement, such as psyllium, that firms loose stool.

Biofeedback

Therapy that uses a sensor to help retrain the pelvic floor and sphincter muscles.

Sacral neuromodulation

A small implant that sends mild signals to the nerves that control the bowel.

Is there really anything that helps? Yes. Many people improve with simple steps, like firming the stool and retraining the muscles. Other options can help when these are not enough. The hardest part is bringing it up. Once you do, your team can make a plan with you.

How It's Diagnosed

- Questions about how often you leak, how much, and what sets it off
- An exam of the anal sphincter and pelvic floor
- Sometimes an ultrasound of the sphincter, pressure tests of the anus and rectum, or a scope

How It's Treated

- 1 Stool control:** take fiber to firm the stool, treat diarrhea or constipation, and keep a regular bowel routine.
- 2 Medicine** such as loperamide when stool is loose.
- 3 Pelvic floor therapy or biofeedback** to strengthen and coordinate the muscles.
- 4 If these are not enough:** sacral neuromodulation, or surgery to repair a torn sphincter. The benefit of repair often fades over the years.
- 5 Limited options:** a gel injected near the anus (bulking) is not routinely advised. Its benefit is small and may not last.

Living With It

- Aim for **soft, formed stool** with fiber and steady fluids.
- Go at regular times. Protect your skin and carry supplies.
- Notice foods that make leakage worse, such as caffeine or spicy foods.

Call the office that is treating you if you have:

- **Blood in the stool**, or black, tarry stools
- A new, big change in bowel habits, or weight loss you cannot explain
- Hard stools or no bowel movement for days while taking loperamide

Go to an emergency department if:

- You have heavy bleeding from the rectum, or feel faint
- You have severe belly pain with fever or vomiting
- You cannot reach the office and are worried

Three Things to Remember

- 1.** Bowel leakage is common and treatable. You do not have to live with it or stay silent.
- 2.** Treatment starts with firming the stool and retraining the muscles.
- 3.** Telling your team is the key first step.